

The Leavell lecture¹—The end of public health as we know it: constructing global public health in the 21st century

This article is a reprint. It was originally published in the journal *Public Health*, Volume 118, Issue 7, Pages 459-538 (October 2004).

Health lies at the core

Health lies at the very core of modernity and development. It has shaped the nature of the modern nation state and its social institutions. Health has powered social movements, defined rights of citizenship and contributed to the construction of the modern self and its aspirations in the developed world—and is increasingly gaining a similar role in developing countries. The success of public health has changed the very nature of developed societies. They have become *health societies*, defined by five major characteristics:

- a high life expectancy and ageing populations;
- an expansive health and medical care system;
- a rapidly growing private health market
- health as a dominant theme in social and political discourse;
- health as a major personal goal in life.

Each of these five characteristics, and perhaps even more their synergies, are changing the face of public health and the extent of its remit. Two key dimensions emerge as driving forces: *do-ability* and expansion of territory and with it a significant number of new public health actors. As in the 19th and 20th centuries, public health action will define progress not only for health but for the social and economic systems of 21st century society. How will we treat the old? How will we pay for health? Who

has a right to care? To what extent will we enhance our biological capabilities? All these questions are not just about health, they are about our way of life, about social justice and about solidarity. The political dimensions of these developments are only beginning to be understood in their full implications, and their impact is no longer limited to the developed world. Increasingly public health is challenged to:

- act in new policy arenas: trade, industry, international law, foreign policy to name but a few;
- move beyond the classic areas of action into fields such as drug; treatment, genetics and human rights;
- respond to the impacts of ever increasing privatization and commercialization of health and health services throughout the world.

The two public health revolutions that changed the face of health and disease in the industrialized countries in the 19th and 20th centuries, the control of infectious disease through protective health measures and the consequent battle against non-communicable disease, are still underway in differing degrees throughout the world. They are intertwined in new ways in many developing countries that now face new patterns of health and disease as well as longer life expectancy without the time and the resources to respond. And finally, with each of the public health revolutions come not only a change in disease patterns but also a significant cognitive shift that implies that health and disease are no longer natural states. Health has become *do-able*; solutions exist, be they medical, economic or social. With the advent of antiretroviral drugs, for example, HIV/AIDS is no longer a death sentence. The *do-ability* of treatment brings issues of availability and access to the fore, for the rich as well as the poor. It is this *do-ability of health* that increasingly drives the debate on human dignity, equity and social justice and raises new challenges for public health action.

The post-modern health societies of the developed world stand in stark contrast to the situation in the poorest countries. Poor countries (and poor excluded populations in more developed regions) face a stark reality:

- a falling life expectancy in many African countries and some Eastern European Countries;
- a lack of access to even the most basic services;
- an excess of personal expenditures for health of the poorest;
- health as a neglected arena of national and development politics;
- health as a matter of survival.

The comparative statistics for maternal and child mortality for Canada and Haiti, two countries that are just a plane ride away in the same region of the WHO, are striking. According to the Pan-American Health Organization, the infant mortality rate is 5.1 in Canada and 97.1 in Haiti, and the under-five mortality rate is 16.5 times greater in Haiti than in Canada. The rate of maternal deaths is 35 times higher in Latin America and the Caribbean than in North America. The 40-year difference between the average life expectancy in Somalia and in Japan becomes unacceptable in the face of the *do-ability* of health. The public health focus on equity, access and health determinants must become increasingly global and examine more systematically how the

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¹ The Hugh R. Leavell Lecture was established as a memorial to Professor Hugh R. Leavell, the first executive secretary of WFPMA, to be a highlight of the conference. The 2004 Lecture is presented by Dr Ilona Kickbusch, one of today's most articulate and internationally recognised public health leaders. Past lecturers have included Dr Zafarullah Chowdhury of Bangladesh, Dr Abraham Horowitz, former Director Emeritus of the Pan American Health Organisation, Dr Nafis Sadik, former Executive Director of the United Nations Population Fund, and James P. Grant, former Executive Director of the United Nations Children's Fund.

Key words

- public health
- Leavell lecture
- governance

public health response must be shared between the rich and the poor, between the international community and the nation states, the individual citizen and the state, the public and the private sector. It must develop new principles for public health action in a global world.

How committed is the developed world?

Health is on the global agenda. In the year 2000, the United Nations at its Millennium Summit adopted the Millennium Development Declaration,¹ a set of eight goals to fight global poverty. A recent analysis by the Global Governance Initiative of the World Economic Forum, came to the conclusion that "the world is doing barely a third of what is necessary to fulfill the goals it has set."² The Initiative gives the world a score of four out of ten for the direct health-related Millennium Development Goals (MDGs), which aim to accomplish the following by 2015:

- stop and begin to reverse the spread of HIV/AIDS, tuberculosis and other diseases;
- reduce by two thirds the under-five mortality rate;
- reduce by three quarters the maternal mortality ratio.

There is an increasing consensus that in order to affect change, development aid is necessary but totally insufficient and in need of a major overhaul. An index developed by the Center for Global Development, called the Commitment to Development Index, ranks 21 of the world's richest countries based on their commitment to broader policies that benefit the 5 billion people living in poorer nations. The index brings together seven issues: aid, trade, investment, migration, environment, security and technology and states: "no wealthy country lives up to its potential to help poor countries. Generosity and leadership remain in short supply."³

Of course it is not only the international community that is at fault. In relation to HIV/AIDS, the report of the Global Governance Initiative puts the blame squarely at the door of *the lack of support and leadership from the governments of affected countries.*⁴ But bad governance in developing countries notwithstanding, the overall history of development aid has contributed

significantly to the present situation, from the approaches to aid as an instrument of the cold war, to the Bretton Woods enforcement of structural adjustment, to fads and fashions in the development business itself. Perverse effects of the system are increasing. For example, one of the unintended consequences of the influx of money for HIV/AIDS treatment and prevention is that doctors and nurses in Africa are abandoning the public sector in order to pursue internationally funded positions in HIV/AIDS programmes. Another is the lack of coordination of foreign aid, for example, a small country like Tanzania was faced with coordinating 1371 different projects funded by international aid agencies between 2000 and 2002.⁵

The expansion of actors and territory in global health

Just as the do-ability of health is one of the key driving forces of global health development so is its *continuous expansion of actors and territory*. At first sight it would seem that there is no reason to be concerned. Health issues have increased on agendas at all levels, from the foreign policies of nations to deliberations within the UN Security Council and General Assembly. Health has the richest foundation in the world—the Bill and Melinda Gates Foundation—on its side. The contribution to health in international donor aid has increased, particularly through the increased contributions to HIV/AIDS. With WHO, health has an intergovernmental agency that encompasses nearly all countries of the world.

The global health area has been transformed by a proliferation of actors, as indicated by the growth of civil society organizations (CSOs), the rise of transnational companies (TNCs), and the increasing involvement in health by organizations, such as the World Bank, regional development banks and regional organizations like the European Union. Since the 1980s, United Nations (UN) agencies besides WHO, such as UNICEF, UNDP, and UNFPA, have increasingly been dealing with health issues. This has been reinforced by a number of UN Summits that have included health goals in their major recommendations. New organizations, such as the Joint UN Program on HIV/AIDS (UNAIDS) and the Global Fund to Fight AIDS, Tuberculosis

and Malaria have been created to take on health matters. Today a health programme run only by one organization is becoming the exception rather than the norm.

Health also has a multitude of institutions, partnerships and alliances, and it has spearheaded new ways for the public and private sector to work together. The UN Global Compact aims to ensure respect for human rights through the integration of such rights in business operations. Furthermore, UN agencies are increasingly involved in health initiatives that are conducted in partnership with many players and organizations. Finally health has benefited from the activists that have put health issues such as the access the antiretroviral drugs at the top of the anti-globalization agenda.

At the same time, there has been a *continuous expansion of the territory of health*. The Alma Ata Declaration opened the door to understand health in the context of development and reinforced the understanding of the first public health revolution that much of the most influential action to create health is found in sectors other than health. The notion of inter-sectoral action was the beginning of a new focus on health determinants. Over the last 20 years, research on socio-economic determinants of health has expanded and reinforced that the health care sector is only one of many sectors (i.e. transportation, housing, education, environment, etc.) that both affect and are affected by health.^{6, 7, 8, 9 and 10}

The WHO approach to health promotion reinforced this trend through programmes such as Healthy Cities. Most recently, the Report of the Commission on Macroeconomics and Health, 'Macroeconomics and Health: Investing in Health for Economic Development', underlined that health is a central component of poverty reduction and economic development of nations.¹¹ It brought the health agenda firmly into the economic development agenda and close to ministries of finance. This was further strengthened by the prominence awarded to health in the MDGs. Yet the public health sector seems ill prepared to take a leading role in this very active arena which—because of the central role of health politically and economically—is increasingly defined by political rather than technical agendas.

The perfect storm

Beneath all this movement lies a serious global health crisis. It has six dimensions, which in turn represent the key challenges in health in the world during the next 10 years. While each of these dimensions is worrying enough in itself, the synergy between the six dimensions is creating 'the perfect storm'. The weakening of public health at all levels of governance that has occurred over the last 30 years means that the public health establishment is not well prepared to deal with these major seminal trends occurring in relation to health and society. Indeed it raises the issue of to what extent the prevailing public health structures and policies can respond adequately to a totally new situation.

Dimension 1: the growth of epidemics

All social and economic development efforts manifest themselves in health outcomes, most importantly in better health and life expectancy. Increasingly, it is understood that health, in itself, is an important development factor.¹² But increasingly there is no us and them any more in relation to infectious and non infectious diseases—both hit the developed and the developing world. The global AIDS epidemic confirms this. Societies that are losing their most productive adult population are also in danger of losing their stability and social cohesion. The economic impacts of the short-lived Severe Acute Respiratory Syndrome (SARS) epidemic in Canada and Asia have been significant. It is estimated that the economic toll of SARS has already reached \$30 billion, largely from cancelled travel (thus impacting the service industry and airlines) and decreased investments.¹³

The developing world is not safe from the health threats of modern lifestyles, as illustrated by the global spread of the tobacco and obesity epidemics. Tobacco is responsible for about 5 million deaths each year, particularly among poor populations and countries. The obesity epidemic is spreading to low- and middle-income nations. WHO points to economic growth, modernization, urbanization, and globalization of food markets as some of the factors contributing to the epidemic.¹⁴ Approximately 177 million people are currently living with diabetes, and the

number of people with the disease is projected to more than double by 2030, especially in developing nations.¹⁵

Dimension 2: the lack of sustainable health systems

The performance of health systems is a crucial component to promoting health and preventing disease. Failures of health systems disproportionately impact the poor, as they are given less respect, less choices of providers and lower quality amenities. The poor suffer from a lack of health care coverage and are forced to pay for their treatment, as privatization of health care spreads throughout the world. In India, for example, families pay 80% of their health costs out-of-pocket, compared to those in industrialized countries with universal health care who pay only 25% on average (with the US as an exception, where the average is 56%). Private sector provision of health care has grown in part due to lack of funding for public health services. According to WHO, nearly 20% of member states spend less than US\$ 15 per capita on health, and health expenditures by the government are often heavily weighted towards tertiary care.¹⁶

Shortages of human resources plague the health systems of the developing world. The lack of health workers is impeding progress towards national health goals and the MDGs, particularly in sub-Saharan Africa. In Botswana, for instance, insufficient health personnel have hindered free distribution of HIV/AIDS drugs. 'Brain drain' of health workers is weakening already fragile health systems. Although research on this migration is in its early stages, policy makers have identified it as a key impediment to a well functioning health system.¹⁷ In addition to human resources, health systems are also hindered by insufficient national capacities for public health in both rich and poor countries.

Dimension 3: the socio-economic-political context

Globalization greatly impacts the socio-economic-political context of health. At present, the poorest countries are feeling the devastating effects of global health disparities, but there are mounting signals that a new health divide is in the making in the developed world. Indeed, it is becoming increasingly difficult to define the rich and the poor of this world at the level of the nation state, as a large

global underclass spreads out around the globe and defies old definitions of vulnerable groups. HIV/AIDS is only the most visible of the diseases of poverty that undermine the life chances of the poor. Developed countries are experiences cut back in services and public health programmes and the European model of social welfare states has come under significant attack. The nation state is less willing to regulate unhealthy goods and services with the consequence that the health society is an increasingly unequal society in terms of access to both prevention and treatment. If concepts of solidarity and universal access are weakened in the developed world, the chances for the developing world to benefit from global partnerships is significantly reduced.

Dimension 4: the values

The value base of global health action has become increasingly vague and unclear. The Health for All orientation of the 1980s was replaced with an investment in health perspective in the 1990s, headed by the World Bank, which brought an economic investment paradigm to the centre of the health debate. Since the end of the Cold War, health has become an integral part of the poverty reduction and social safety net strategies of the international community. In paradigmatic terms, this means that the acceptance of health as an end (as reflected in the human rights approach) has been overshadowed by the approach to health as a means.

There have been new attempts to embark on a debate as to which values should drive global health action. These values are reflected in discussions around the impact of globalization, human rights, global public goods (GPGs) and global social contracts. The debate on new global financing mechanisms is basically one about the values: According to what principles should wealth be shared at the global level?

Dimension 5: the global actors

The new interactions among global health players is transforming the global playing field in health—its norms, rules, practices and, especially, its power politics.¹⁸ The shift from state-centred politics to more complex forms of governance and the increased number of players and collaborative arrangements in global health have led many to believe

that there has been a diffusion of power among global health players. This is not necessarily the case. The increased interconnectedness between health and non-health actors is blurring their traditional boundaries,¹⁹ and the permeability of national borders has diminished governmental control over a growing number of health determinants.²⁰ At the same time Member States have systematically weakened the remit of their own intergovernmental organization, the World Health Organization.

In consequence, accountability and transparency have emerged as key issues among global health actors. However, few experts have examined its role in health,²¹ and definitions of accountability are often vague. Actors in the public, business and civil society sectors differ in to whom they are accountable. Indeed globalization may lead to a reduction of accountability.²² Brugha and Zwi, for example, show that accountability was lacking at both national and global levels when the World Bank and USAID pushed for privatization of health care—policy advice that later was regarded as questionable at best.²³ Furthermore, lack of transparency in creating policies only worsens the already inadequate accountability of powerful global organizations.²⁴ Negotiations at the WTO on public health issues have not been transparent nor have they included the voice of civil society. Instead they have catered to the interests of industrialized nations' commercial sector. The dispute resolution process, held behind closed doors, has not incorporated the technical expertise of public health experts but instead relied on trade lawyers and diplomats.²⁵ Finally, global health partnerships also face challenges in accountability because the role and responsibilities of each partner are not always clear.

Dimension 6: systems failure

The development agencies and lending institutions have not been willing to support what laid the basis for the health and life expectancy gains in the first and second public health revolutions: a strong state, laws and regulation, public health, public education and the understanding that health is part and parcel of a citizen's right. In the 19th century with the social conflict around health and citizen's rights, a new

principle entered health governance: the concept of solidarity as an integrative force for both social movements and for identity and cohesion within the nation state. Public health was understood to be a social enterprise.

This collective and societal orientation of public health has been lost. Yet in the face of globalization Geoffrey Rose's public health dictum is more important than ever: "The primary determinants of disease are mainly economic and social, therefore its remedies must also be economic and social."²⁶

One element of the systems failure in global health has been the tendency to be wedded to a *charity model*, which focused on the 'deserving' and the 'undeserving' poor rather than on citizens rights. 'Global health means that the health of the poorest and most vulnerable has direct relevance for all populations because of the many interconnectivities that increasingly bring the world closer. From this perspective, the underlying basis of health sector aid should shift from providing charitable handouts to ensuring appropriate and sufficient resources for a global health system that meets the common needs of the human species.'²⁷

A new conceptual map

Public health is at a crossroads. The changes that societies are facing are as significant as the ones encountered in the 'golden era' of public health 150 years ago, and they are truly global in nature. In consequence, a new conceptual map is needed for public health action that incorporates, in new ways, scientific and technological development; political, social and economic action; and domestic and global public health responsibilities. Inge Kaul, in her work on GPGs, emphasizes the need to develop a new global policy model. "The pervasiveness of today's crises suggests that they might all suffer from a common cause, such as a common flaw in policy making, rather than issue specific problems. If so, issue specific responses typical to date, would be insufficient—allowing global crises to persist and even multiply."²⁸

It was one of the characteristics of modernity to take health out of the confines of religion and charity and make

it a key element of state action and the rights of citizenship. This process, initially within the context of the constitution of the nation state, today needs to go global as a key dimension of global justice. The present global drive for access to AIDS medicines for developing nations, for instance, is not just about health. It is the spearhead of a global citizenship movement that has recognized that global health needs to move out of the charity mode into the realm of rights, citizenship and a global contract. The global public health community must reorient and strengthen public health within both developed and developing societies as a joint endeavour, and advocate for a resilient system of global governance for health based on a new global social contract, a point also raised by Richard Smith in a recent BMJ editorial.²⁹

The third public health revolution is global

The approach to public health must change—it must again become an art and a science, it must be a discipline that is fully engaged in the political and social arena. There is more at stake than just health—because the new interdependence and the new global dynamics have positioned health as a defining characteristic of the global society of the 21st century. The third public health revolution needs to be a global revolution oriented around five principle areas of action.

Health as a global public good

Securing global population health must, first and foremost, be seen as a GPG. GPGs are defined as having *non-excludable, non-rival benefits that cut across borders, generations, and populations*.³⁰ There is no us and them any more in health. The GPG concept implies that society must ensure the value of health, understand it as a key dimension of global citizenship and keep it high on the global political agenda. It means defining common agendas, increasing the importance of global health treaties and pooling of sovereignty by nation states in the area of health. 'The GPG perspective demonstrates that today's global health challenges require not just good national policies, but also strong global responses, the focal point thus being *international collective action*.'³¹

Health as a key component of global security

Global integration has proven that disease outbreaks are a threat to national, international and human security. Nation states should incorporate health issues into their security and their development strategies. They need to understand that in health domestic and foreign policy increasingly overlap. Global health surveillance and rapid response must be ensured and the WHO will

need to extend its global health surveillance role and expand its interventionist power in the context of the International Health Regulations. The WHO member states and the private sector must also comply with international bodies in reporting potential health threats and should support the joint financing of a global surveillance infrastructure. A rapid health response force could be ensured through a new kind of GPGs tax, for example, on airline travel.

Strengthening global health governance for interdependence

Health must not only be viewed through the lens of development but also through the reality of interdependence. WHO must use its existing constitutional capability to ensure agenda coherence in global health and it must strengthen its convening capabilities. It should further develop mechanisms that ensure transparency and accountability in global health governance and play a brokering role in relation to the health impacts of

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